

SUPERVISED VOCATIONAL QUALIFICATION DIPLOMA - EXAMINATION

Session: Revision Mock Set 08	Duration: 3 hours
Specialty: Psychotherapy	Coefficient: 04
Eliminatory mark: < 8/20	Training reference: VTI-AEDA examination style

COMPREHENSIVE VOCATIONAL PAPER - SOLUTION / MARKING GUIDE PRACTICE SET 08

Revision material based on the supplied VTI-AEDA 2025 examination format and the supplied psychotherapy course recordings. Not an official examination paper.

How to use this marking guide

For revision, first attempt the paper without notes. Then compare not only the answer letter but the clinical cue that makes the answer correct. For legal/ethical items, the guide follows the concepts emphasized by the supplied course/examination materials; in real practice, applicable Cameroon law, institutional policy and the current professional code must also be checked.

PART ONE: PSYCHOPATHOLOGY - ANSWERS

1. During assessment, Grace reports excessive need to be cared for, difficulty making decisions independently, submissiveness and fear of separation. Which of the following is the BEST match?

- A. Schizoid personality disorder
- B. Dependent personality disorder**
- C. Stimulant intoxication
- D. Borderline personality disorder

Answer: B - Dependent personality disorder. Dependent personality is characterized by reliance on others for decisions, reassurance and support.

Memory aid: *DEPENDENT = Decisions Entrusted to People; separation fear.*

2. Amina, a 45-year-old accountant, presents with persistent social-communication difficulties together with restricted or repetitive patterns of behavior or interests beginning in development. Which diagnosis or clinical concept best fits this presentation?

- A. Borderline personality disorder
- B. Major depressive disorder
- C. Bipolar disorder
- D. Autism spectrum disorder**

Answer: D - Autism spectrum disorder. Autism requires both social-communication differences and restricted/repetitive behavior patterns.

Memory aid: *2R domains: Relating/communication + Repetitive/restricted behavior.*

3. Grace, a 29-year-old laboratory technician, presents with recurrent unexpected panic attacks followed by persistent worry about more attacks or maladaptive avoidance. Which diagnosis or clinical concept best fits this presentation?

- A. Schizoid personality disorder
- B. Specific phobia
- C. Panic disorder**
- D. Obsessive-compulsive disorder (OCD)

Answer: C - Panic disorder. Panic disorder is more than a single panic attack: recurrent unexpected attacks are followed by ongoing concern or behavior change.

Memory aid: *PANIC: Panic Attacks + New worry/avoidance Is Continuing.*

4. A therapist notes that Amina has excessive, difficult-to-control worry across several life areas for about six months, with tension, restlessness or poor sleep. Which condition should be considered first from the options below?

- A. Major depressive disorder

B. Anorexia nervosa, restricting type

C. Generalized anxiety disorder (GAD)

D. Panic disorder

Answer: C - Generalized anxiety disorder (GAD). GAD is characterized by persistent generalized worry across multiple domains rather than fear tied to one object or situation.

Memory aid: GAD = General Anxiety across Domains.

5. Victor, a 52-year-old laboratory technician, presents with marked fear of a specific object or situation that triggers immediate anxiety and avoidance out of proportion to the actual danger. Which diagnosis or clinical concept best fits this presentation?

A. Specific phobia

B. Generalized anxiety disorder (GAD)

C. Social anxiety disorder

D. Borderline personality disorder

Answer: A - Specific phobia. A specific phobia is circumscribed: the fear is linked to a particular object or situation and typically produces avoidance.

Memory aid: SPECIFIC = one SPECIFIC trigger.

6. Victor, a 32-year-old sales representative, presents with marked fear of social or performance situations because of possible scrutiny, embarrassment or negative evaluation. Which diagnosis or clinical concept best fits this presentation?

A. Major depressive disorder

B. Social anxiety disorder

C. Opioid intoxication

D. Post-traumatic stress disorder (PTSD)

Answer: B - Social anxiety disorder. Social anxiety centers on being observed, judged or embarrassed in social/performance settings.

Memory aid: SOCIAL = Scrutiny Of Client In Activities with Lots of people.

7. A therapist notes that Zachary has fear and avoidance of situations where escape may be difficult or help unavailable, such as crowds, public transport or being outside alone. Which condition should be considered first from the options below?

A. Post-traumatic stress disorder (PTSD)

B. Agoraphobia

C. Bulimia nervosa

D. Social anxiety disorder

Answer: B - Agoraphobia. Agoraphobia concerns situations in which escape or access to help feels difficult; it may occur with or without panic disorder.

Memory aid: AGORA = Afraid to Go Out/Risk being Away from help.

8. Uma, a 27-year-old laboratory technician, presents with intrusive unwanted thoughts or urges and/or repetitive behaviors or mental acts performed to reduce anxiety. Which diagnosis or clinical concept best fits this presentation?

A. Anorexia nervosa, restricting type

B. Agoraphobia

C. Obsessive-compulsive disorder (OCD)

D. Brief psychotic disorder

Answer: C - Obsessive-compulsive disorder (OCD). OCD involves obsessions (intrusive thoughts/urges/images) and compulsions (repetitive acts or mental rituals).

Memory aid: O = Obsession is a thought; C = Compulsion is a Conduct.

9. During assessment, Esther reports after trauma, recurrent intrusions or nightmares, avoidance, negative changes in mood/cognition and heightened arousal persisting beyond one month. Which of the following is the BEST match?

A. Post-traumatic stress disorder (PTSD)

B. Borderline personality disorder

C. Anorexia nervosa, restricting type

D. Substance use disorder

Answer: A - Post-traumatic stress disorder (PTSD). PTSD follows exposure to trauma and is organized around intrusion, avoidance, negative cognitive/mood changes and arousal/reactivity.

Memory aid: IANA: *Intrusion, Avoidance, Negative changes, Arousal.*

10. A therapist notes that Nadine has at least two weeks of depressed mood and/or loss of interest with associated sleep, appetite, energy, concentration, guilt, psychomotor or suicidal symptoms. Which condition should be considered first from the options below?

A. Major depressive disorder

B. Schizoid personality disorder

C. Schizophrenia

D. Stimulant intoxication

Answer: A - Major depressive disorder. Major depression is a sustained depressive syndrome, not simply transient sadness. Functional impairment and risk, especially suicidality, must be assessed.

Memory aid: SIGECAPS: *Sleep, Interest, Guilt, Energy, Concentration, Appetite, Psychomotor, Suicide.*

SECTION B: TRUE/FALSE - EXPLANATIONS

1. FALSE. PTSD is diagnosed solely because a frightening event occurred.

Trauma exposure is necessary but not sufficient; a characteristic symptom pattern and duration/impact are also required.

2. TRUE. Dependent personality disorder may involve difficulty making everyday decisions without reassurance.

Excessive reliance on others is a central feature.

3. TRUE. An opioid is more likely than a stimulant to produce drowsiness and physiologic slowing.

Opioids commonly produce sedation, whereas stimulants activate.

4. FALSE. A manic episode is simply a happy mood with no change in energy or functioning.

Mania involves abnormal mood plus increased energy/activity and additional symptoms with significant impact.

5. TRUE. Anorexia nervosa can include binge-eating/purging behavior if the person also meets the low-weight anorexia criteria.

That is the binge-eating/purging subtype of anorexia nervosa.

SECTION C: MODEL SHORT ANSWERS

1. Differentiate anorexia nervosa from bulimia nervosa. (2mks)

- Anorexia requires significantly low body weight with restriction/fear of weight gain/body-image disturbance.

- Bulimia centers on recurrent binge eating plus compensatory behavior; markedly low weight is not required.

Memory aid: *Low weight points strongly toward anorexia.*

2. State two characteristic features of a manic episode. (2mks)

- Reduced need for sleep, grandiosity, pressured speech, flight of ideas, increased goal-directed activity, risky behavior - any two.

Memory aid: DIG FAST.

3. What is comorbidity? (2mks)

- The presence of two or more disorders in the same person at the same time or over an overlapping period.

Memory aid: *CO-morbidity = conditions CO-occur.*

4. Name the four broad PTSD symptom clusters taught in modern formulations. (2mks)

- Intrusion

- Avoidance

- Negative changes in cognition/mood

- Arousal/reactivity

Memory aid: IANA.

5. What is meant by anhedonia? (2mks)

- Marked loss of interest or pleasure in activities previously enjoyed; a core depressive symptom.

Memory aid: *AN-hedonia = absence of pleasure.*

PART TWO: THEORIES/TECHNIQUES - ANSWERS

1. In a session with Amina, the counselor emphasizes behavior is shaped by its consequences, including reinforcement and punishment. This BEST illustrates:

- A. Flooding
- B. Psychoanalytic therapy
- C. Exposure therapy
- D. Operant conditioning**

Answer: D - Operant conditioning. Operant conditioning focuses on consequences that increase or decrease behavior.

Memory aid: Operant = behavior OPERATES, consequence follows.

2. In a session with Nadine, the counselor emphasizes a behavior increases because it removes or prevents an aversive stimulus. This BEST illustrates:

- A. Negative reinforcement**
- B. Dialectical Behavior Therapy (DBT)
- C. Resistance
- D. Existential therapy

Answer: A - Negative reinforcement. Negative reinforcement strengthens behavior; negative means something unpleasant is removed, not that behavior is punished.

Memory aid: Reinforcement always raises behavior; negative removes something.

3. A therapist working with Yvette uses an intervention characterized by a previously reinforced behavior decreases when the reinforcement maintaining it is consistently withheld. Which approach or technique is being used?

- A. Extinction**
- B. Existential therapy
- C. Classical conditioning
- D. Empty-chair technique

Answer: A - Extinction. Extinction occurs when reinforcement is no longer provided for a behavior.

Memory aid: Extinction = extinguish the payoff.

4. Which psychotherapy concept is most accurately described by the following: learning behavior by observing what happens to another person or by imitating a model?

- A. Cognitive restructuring
- B. Countertransference
- C. Social modeling**
- D. Gestalt dream work

Answer: C - Social modeling. Modeling is observational learning: the client learns from another person's behavior and its consequences.

Memory aid: MODEL = learn by watching a model.

5. Which psychotherapy concept is most accurately described by the following: planned contact with feared stimuli, memories or situations while reducing avoidance so fear can be relearned?

- A. Rational emotive imagery
- B. Rational Emotive Behavior Therapy (REBT)
- C. Exposure therapy**
- D. Resistance

Answer: C - Exposure therapy. Exposure reduces avoidance and allows corrective learning; it may be in vivo, imaginal or other formats.

Memory aid: Approach, do not avoid.

6. In a session with Brenda, the counselor emphasizes direct, real-life exposure to a feared object or situation. This BEST illustrates:

A. In vivo exposure

- B. Cognitive behavioral therapy (CBT)
- C. Social skills training
- D. Retroreflection

Answer: A - In vivo exposure. In vivo means exposure in the actual situation rather than only in imagination.

Memory aid: VIVO = in real life.

7. A therapist working with Hassan uses an intervention characterized by gradual exposure to a fear hierarchy combined with relaxation or another incompatible response. Which approach or technique is being used?

- A. Cognitive behavioral therapy (CBT)
- B. Exposure therapy
- C. Systematic desensitization**
- D. Adlerian therapy

Answer: C - Systematic desensitization. Systematic desensitization classically combines relaxation training with stepwise movement through a fear hierarchy.

Memory aid: RHE: Relax, Hierarchy, Expose gradually.

8. A therapist working with Tina uses an intervention characterized by rapid and prolonged exposure to a high-intensity feared stimulus or situation without gradual hierarchy progression. Which approach or technique is being used?

- A. Free association
- B. Negative reinforcement
- C. Psychoanalytic therapy
- D. Flooding**

Answer: D - Flooding. Flooding differs from graded exposure because the client confronts a high level of fear more rapidly.

Memory aid: Flooding = jump into the deep end.

9. A therapist working with Daniel uses an intervention characterized by systematically tensing and releasing muscle groups to learn physical relaxation and reduce arousal. Which approach or technique is being used?

- A. Social modeling
- B. Progressive muscle relaxation**
- C. Gestalt therapy
- D. Rational Emotive Behavior Therapy (REBT)

Answer: B - Progressive muscle relaxation. PMR builds awareness of tension and relaxation by working through muscle groups.

Memory aid: Tense - notice - release - breathe.

10. A therapist working with Irene uses an intervention characterized by a skills-based therapy especially associated with emotion dysregulation and borderline personality disorder, using mindfulness, distress tolerance, emotion regulation and interpersonal effectiveness. Which approach or technique is being used?

- A. Dialectical Behavior Therapy (DBT)**
- B. Adlerian therapy
- C. Free association
- D. Retroreflection

Answer: A - Dialectical Behavior Therapy (DBT). DBT combines acceptance and change strategies and teaches concrete skills for severe emotion dysregulation.

Memory aid: M-D-E-I: Mindfulness, Distress tolerance, Emotion regulation, Interpersonal effectiveness.

11. Which psychotherapy concept is most accurately described by the following: behavioral teaching and rehearsal of interpersonal skills such as greetings, eye contact, listening and conversation?

- A. Cognitive restructuring
- B. Social skills training**

- C. Dialectical Behavior Therapy (DBT)
- D. Rational Emotive Behavior Therapy (REBT)

Answer: B - Social skills training. Social skills training uses instruction, modeling, rehearsal and feedback to build interpersonal behavior.

Memory aid: *IMRF: Instruction, Modeling, Rehearsal, Feedback.*

12. In a session with Samuel, the counselor emphasizes the client says whatever thoughts, feelings, images or memories come to mind without censoring them. This BEST illustrates:

- A. Systematic desensitization
- B. Free association**
- C. Existential therapy
- D. Dialectical Behavior Therapy (DBT)

Answer: B - Free association. Free association is a classic psychoanalytic technique intended to reduce censorship and reveal unconscious material.

Memory aid: *Psychoanalytic FIRT: Free association, Interpretation, Resistance, Transference.*

13. In a session with Kemi, the counselor emphasizes the therapist explains or offers meaning about patterns, defenses, dreams, resistance or behavior. This BEST illustrates:

- A. Interpretation**
- B. Cognitive behavioral therapy (CBT)
- C. Adlerian therapy
- D. Cognitive restructuring

Answer: A - Interpretation. Interpretation is the analyst's attempt to make latent or previously unconscious meaning more understandable to the client.

Memory aid: *Interpretation = therapist offers possible meaning.*

14. A therapist working with Patricia uses an intervention characterized by the client unconsciously avoids material or processes that threaten to bring unacceptable conflict into awareness. Which approach or technique is being used?

- A. Resistance**
- B. In vivo exposure
- C. Transference
- D. Progressive muscle relaxation

Answer: A - Resistance. Resistance is the defensive blocking of threatening material or therapeutic work.

Memory aid: *Resistance = psychological brakes.*

15. A therapist working with Fidel uses an intervention characterized by the client redirects feelings, expectations or relationship patterns from important past relationships onto the therapist. Which approach or technique is being used?

- A. Transference**
- B. Flooding
- C. Existential therapy
- D. Gestalt therapy

Answer: A - Transference. Transference originates primarily from the client's earlier relationships and becomes enacted in the therapeutic relationship.

Memory aid: *TRANSference = client TRANSfers old relationship feelings to therapist.*

SECTION B: TRUE/FALSE - EXPLANATIONS

1. FALSE. Progressive muscle relaxation involves systematically increasing tension and never releasing it.

The method alternates tension with deliberate release and relaxation.

2. FALSE. In vivo exposure occurs only in imagination.

In vivo exposure takes place in the real feared situation or with the real feared object.

3. **TRUE.** Retroflexion involves directing toward oneself an impulse that might otherwise be directed outward.

That is the Gestalt meaning of retroflexion.

4. **TRUE.** Rational emotive imagery belongs to the REBT family of techniques.

It is used to rehearse changing unhealthy emotion by changing beliefs.

5. **TRUE.** Social skills training can use modeling, role play, rehearsal and feedback.

Those are common behavioral teaching components.

SECTION C: MODEL SHORT ANSWERS

1. What is transference? **(2mks)**

- Client projects/re-enacts feelings, expectations or patterns from earlier important relationships onto the therapist.

Memory aid: Client TRANSfers old patterns.

2. What is countertransference? **(2mks)**

- Therapist emotional response to the client, especially where the therapist's own history or vulnerabilities shape that response; requires awareness/supervision.

Memory aid: Counter = counselor side.

3. State one goal of existential therapy. **(1mks)**

- Help the client face freedom/responsibility, clarify meaning and values, make authentic choices and live more deliberately - any one.

Memory aid: FMRC.

PART THREE: ETHICAL/LEGAL - ANSWERS

1. During supervision, Fidel describes keeping commitments, maintaining trust and being faithful to professional responsibilities. Which concept should guide the discussion?

- A. Ethical response to colleague misconduct
- B. Privacy
- C. Fidelity**
- D. Veracity

Answer: C - Fidelity. Fidelity concerns loyalty, promises and trust within professional relationships.

Memory aid: *Fidelity = faithful to promises.*

2. A counselor asks which principle or professional concept is defined by an ongoing process in which the client receives understandable information about services, risks, benefits, alternatives, limits of confidentiality and choice. The BEST answer is:

- A. Termination and referral
- B. Subpoena response
- C. Informed consent**
- D. Sexual relationship with a current client

Answer: C - Informed consent. Informed consent is not merely a signature; it is an ongoing communication process supporting voluntary, informed participation.

Memory aid: *BRALC: Benefits, Risks, Alternatives, Limits, Choice.*

3. In professional practice, a situation calls for the professional duty to protect information learned in the therapeutic relationship, subject to recognized legal and ethical exceptions. Which ethical/legal concept is MOST directly involved?

- A. Confidentiality**
- B. Justice
- C. Duty to warn/protect
- D. Dual or multiple relationship

Answer: A - Confidentiality. Confidentiality protects client information but is not absolute; limits should be explained early.

Memory aid: *Confidentiality = keep private, know the exceptions.*

4. In professional practice, a situation calls for the client's broader right to control access to self, body and personal information. Which ethical/legal concept is MOST directly involved?

- A. Technology ethics
- B. Termination and referral
- C. Privacy**
- D. Informed consent

Answer: C - Privacy. Privacy is the client's right to be left alone or control access, whereas confidentiality is the professional's duty regarding information received.

Memory aid: *Privacy belongs to the person; confidentiality binds the professional.*

5. A counselor asks which principle or professional concept is defined by a legal protection that may prevent compelled disclosure of certain therapy communications in judicial proceedings, depending on jurisdiction. The BEST answer is:

- A. Informed consent
- B. Sexual relationship with a current client
- C. Privileged communication**
- D. Nonmaleficence

Answer: C - Privileged communication. Privilege is a legal evidentiary concept and is distinct from the broader ethical duty of confidentiality.

Memory aid: *Privilege = courtroom/legal shield.*

6. A counselor asks which principle or professional concept is defined by taking reasonable protective action when a client presents a serious, credible threat toward an identifiable person, as required by applicable law and ethics. The BEST answer is:

A. Duty to warn/protect

- B. Ethical response to colleague misconduct
- C. Fidelity
- D. Dual or multiple relationship

Answer: A - Duty to warn/protect. The course tests duty to warn/protect as an exception to confidentiality when serious danger to others is present. Actual legal requirements vary by jurisdiction.

Memory aid: *TIP: Threat serious, Identifiable target, Protect reasonably.*

7. In professional practice, a situation calls for reporting suspected abuse or other legally reportable situations to designated authorities when law or professional duty requires it. Which ethical/legal concept is MOST directly involved?

A. Mandated reporting

- B. Autonomy
- C. Bartering
- D. Confidentiality

Answer: A - Mandated reporting. Mandated reporting is a recognized limit to confidentiality; therapists should explain this limit during informed consent.

Memory aid: *Report when law makes reporting mandatory.*

8. A counselor asks which principle or professional concept is defined by professional negligence in which a duty is breached and the breach causes harm. The BEST answer is:

- A. Justice
- B. Veracity
- C. Nonmaleficence

D. Malpractice

Answer: D - Malpractice. Malpractice requires more than an unfavorable outcome: there must be a professional duty, breach, harm and causal link.

Memory aid: *DBHC: Duty, Breach, Harm, Causation.*

9. In professional practice, a situation calls for practicing only within areas supported by appropriate education, training, supervised experience and current skill. Which ethical/legal concept is MOST directly involved?

- A. Justice
- B. Termination and referral
- C. Professional competence**
- D. Boundary violation

Answer: C - Professional competence. Using techniques without adequate training can expose clients to harm and the professional to ethical or legal liability.

Memory aid: *Know your scope; seek supervision or refer.*

10. In professional practice, a situation calls for a situation in which the therapist has another significant role or relationship with the client in addition to therapy. Which ethical/legal concept is MOST directly involved?

- A. Dual or multiple relationship**
- B. Bartering
- C. Mandated reporting
- D. Professional competence

Answer: A - Dual or multiple relationship. Multiple relationships are not automatically harmful in every context, but should be avoided or carefully managed when they risk exploitation, impaired objectivity or harm.

Memory aid: *Two roles = review power, objectivity, exploitation and harm.*

11. A counselor asks which principle or professional concept is defined by a departure from customary therapeutic boundaries that may be clinically appropriate and non-exploitative in context. The BEST answer is:

- A. Dual or multiple relationship
- B. Record keeping
- C. Boundary crossing**
- D. Subpoena response

Answer: C - Boundary crossing. A boundary crossing is not automatically unethical; context, culture, intent and effect matter.

Memory aid: Crossing may be thoughtful; violation causes or risks serious harm/exploitation.

12. A counselor asks which principle or professional concept is defined by a serious harmful or exploitative breach of therapeutic boundaries. The BEST answer is:

- A. Record keeping
- B. Sexual relationship with a current client
- C. Bartering
- D. Boundary violation**

Answer: D - Boundary violation. Boundary violations exploit the therapeutic relationship or cause significant harm and are unethical.

Memory aid: Violation = harm/exploitation.

13. A counselor asks which principle or professional concept is defined by sexual or romantic involvement with a current therapy client. The BEST answer is:

- A. Sexual relationship with a current client**
- B. Duty to warn/protect
- C. Confidentiality
- D. Dual or multiple relationship

Answer: A - Sexual relationship with a current client. Sexual relationships with current clients are prohibited because of power imbalance and exploitation risk. Terminating therapy to start a relationship does not erase those concerns.

Memory aid: Current client + sex = no.

14. In professional practice, a situation calls for considering clinical meaning, value, timing, culture, power and exploitation risk before accepting or declining a client gift. Which ethical/legal concept is MOST directly involved?

- A. Privacy
- B. Ethical response to colleague misconduct
- C. Subpoena response
- D. Gift decision-making**

Answer: D - Gift decision-making. Gifts require contextual judgment. A culturally meaningful modest gift may be handled differently from an expensive or coercive gift.

Memory aid: VALUE: Value, Understanding/meaning, Local culture, Unequal power, Effect on therapy.

15. During supervision, Lucas describes accepting goods or services instead of money when ethically permissible, not exploitative and not clinically contraindicated. Which concept should guide the discussion?

- A. Bartering**
- B. Technology ethics
- C. Boundary crossing
- D. Beneficence

Answer: A - Bartering. Bartering may be acceptable under some codes when not exploitative and when arrangements are clear and clinically appropriate.

Memory aid: Barter only if Boundaries and Benefits are safe.

SECTION B: MODEL SHORT ANSWERS

1. Differentiate privacy from confidentiality. (2mks)

- Privacy is the client's right to control access to self and personal information.
- Confidentiality is the professional duty to protect information obtained in the professional relationship.

Memory aid: *Privacy = person's right; confidentiality = professional's duty.*

2. Differentiate confidentiality from privileged communication. (1mks)

- Confidentiality is an ethical/professional duty.
- Privilege is a legal evidentiary protection against compelled disclosure in certain proceedings and depends on jurisdiction.

Memory aid: *Privilege = courtroom shield.*

3. Define malpractice. (1mks)

- Professional negligence in which a duty is breached and the breach causes harm; commonly considered through duty, breach, causation and damages/harm.

Memory aid: *DBHC.*

4. Give two elements of informed consent. (1mks)

- Examples: nature/purpose of services, risks, benefits, alternatives, fees, therapist qualifications, limits of confidentiality, voluntary choice, right to ask questions.

Memory aid: *BRALC.*

SECTION C: MODEL SHORT ESSAYS

1. When is termination ethically appropriate? (2mks)

- When goals are met, treatment is no longer beneficial/needed, the therapist cannot provide competent service, or other legitimate reasons exist - with adequate planning to avoid abandonment.

Memory aid: *PLAN.*

2. State one key ethical requirement for clinical records. (2mks)

- Records should be accurate, timely, relevant and securely stored with access protected.

Memory aid: *ACTS.*

3. Why are dual relationships ethically risky? (1mks)

- They can impair objectivity or professional judgment.
- They can increase risk of exploitation, coercion, conflicts of interest or harm.

Memory aid: *Power + objectivity + exploitation.*

PART FOUR: CULTURE / GROUP / COUNSELING PROCESS / TREATMENT PLAN - ANSWERS

1. In counseling practice, Tina encounters a situation involving implementing agreed therapeutic strategies and monitoring the client's response. Which concept BEST applies?

- A. Intervention stage**
- B. Skills-development group
- C. Performing stage
- D. Open-ended questioning

Answer: A - Intervention stage. Intervention follows assessment and goal definition and should be linked to the case formulation and treatment plan.

Memory aid: Assess -> Goal -> Intervene -> Review.

2. A trainee observes reviewing progress, consolidating coping skills, planning relapse prevention or referral and ending the professional relationship responsibly. This is an example of:

- A. Cognitive-behavioral group
- B. Termination and follow-up**
- C. Intervention stage
- D. Progress review

Answer: B - Termination and follow-up. Termination is a therapeutic phase, not simply stopping appointments.

Memory aid: RECAP: Review, Evaluate, Consolidate, Aftercare, Part.

3. A trainee observes attending closely to verbal and nonverbal communication and showing the client that the message has been heard and understood. This is an example of:

- A. Paraphrasing
- B. Therapeutic confrontation
- C. Cultural competence
- D. Active listening**

Answer: D - Active listening. Active listening includes presence, minimal encouragers, reflection, clarification and appropriate nonverbal attending.

Memory aid: SOLER: Squarely face, Open posture, Lean, Eye contact, Relaxed.

4. A trainee observes identifying and feeding back the emotional meaning expressed by the client. This is an example of:

- A. Reflection of feeling**
- B. Termination and follow-up
- C. Progress review
- D. Group confidentiality

Answer: A - Reflection of feeling. Reflection of feeling helps clients recognize and explore emotions rather than only facts.

Memory aid: Name the feeling, not just the facts.

5. In counseling practice, Daniel encounters a situation involving restating the essence of the client's content in fresh words without changing its meaning. Which concept BEST applies?

- A. Progress review
- B. Self-disclosure
- C. Paraphrasing**
- D. Group confidentiality

Answer: C - Paraphrasing. Paraphrasing checks understanding and encourages elaboration.

Memory aid: Paraphrase = same meaning, new words.

6. In counseling practice, Yvette encounters a situation involving questions that invite elaboration rather than a yes/no answer. Which concept BEST applies?

- A. Reflection of feeling

B. Cultural competence

C. Paraphrasing

D. Open-ended questioning

Answer: D - Open-ended questioning. Open questions support exploration, while excessive questioning can turn therapy into an interrogation.

Memory aid: WHAT/HOW usually open more than DID/IS.

7. A trainee observes respectfully pointing out discrepancies between a client's words, behaviors, values or nonverbal cues. This is an example of:

A. Termination and follow-up

B. Ethnocentrism

C. Therapeutic confrontation

D. Paraphrasing

Answer: C - Therapeutic confrontation. Confrontation is not aggression; it is a careful invitation to examine inconsistency.

Memory aid: Confront discrepancy, not the person.

8. In counseling practice, Grace encounters a situation involving the therapist deliberately shares limited personal information when it is likely to benefit the client rather than meet the therapist's own needs. Which concept BEST applies?

A. Self-disclosure

B. Adjourning stage

C. Open-ended questioning

D. Norming stage

Answer: A - Self-disclosure. Self-disclosure should be purposeful, brief and client-centered.

Memory aid: WAIT: Why am I telling this?

9. In counseling practice, Wendy encounters a situation involving a collaborative written or explicit plan linking identified problems to goals, measurable objectives, interventions and review. Which concept BEST applies?

A. Group role

B. Forming stage

C. Treatment plan

D. Assessment

Answer: C - Treatment plan. Treatment plans organize care around the client's specific needs and should be revised according to progress.

Memory aid: PGOIR: Problem, Goal, Objectives, Interventions, Review.

10. In counseling practice, Samuel encounters a situation involving an organized explanation linking the client's presenting problems with relevant history, triggers, maintaining factors, strengths and a rationale for intervention. Which concept BEST applies?

A. Case formulation

B. Intervention stage

C. Assessment

D. Active listening

Answer: A - Case formulation. Formulation goes beyond a diagnosis by explaining why this person is struggling in this way at this time and what may help.

Memory aid: 5P option: Presenting, Predisposing, Precipitating, Perpetuating, Protective.

11. A trainee observes evaluating whether objectives are being achieved and modifying the treatment plan when progress is absent, circumstances change or goals are met. This is an example of:

A. Intervention stage

B. Cognitive-behavioral group

C. Forming stage

D. Progress review

Answer: D - Progress review. Treatment planning is dynamic; review links evidence of progress to continuation, modification or termination.

Memory aid: Review = measure, discuss, revise.

12. In counseling practice, Quentin encounters a situation involving ongoing development of awareness, cultural knowledge, clinical skills and meaningful encounters with diverse clients. Which concept BEST applies?

A. Professional interpreter use

B. Cultural competence

C. Skills-development group

D. Assessment

Answer: B - Cultural competence. Cultural competence requires reflective awareness plus knowledge, skills and real encounters; it is not achieved by assuming everyone should conform to the therapist's culture.

Memory aid: AKSE: Awareness, Knowledge, Skills, Encounters.

13. A trainee observes judging other cultures primarily by the standards of one's own culture and treating one's own as the norm. This is an example of:

A. Intervention stage

B. Skills-development group

C. Ethnocentrism

D. Reflection of feeling

Answer: C - Ethnocentrism. Ethnocentrism places one's own cultural frame at the center and may distort assessment or communication.

Memory aid: ETHNO-centred = my culture at the centre.

14. Which counseling/group/cultural concept is BEST described by: pressuring a client to adopt the therapist's or dominant culture's beliefs, values or practices?

A. Rapport building

B. Support group

C. Cultural imposition

D. Therapeutic confrontation

Answer: C - Cultural imposition. Cultural imposition occurs when the helper pushes their own values or practices onto the client.

Memory aid: IMPOSITION = impose my culture.

15. Which counseling/group/cultural concept is BEST described by: using a qualified interpreter rather than relying on a child or untrained friend when language barriers affect care?

A. Storming stage

B. Skills-development group

C. Professional interpreter use

D. Progress review

Answer: C - Professional interpreter use. Qualified interpreters improve accuracy, confidentiality and informed consent.

Memory aid: Interpreter = accuracy + confidentiality.

SECTION B: MODEL SHORT ESSAYS

1. What is case formulation? (4mks)

- An organized explanation of how presenting problems connect with history, triggers, maintaining factors and strengths/protective factors.

- It guides treatment by explaining why the problem developed/persists and what interventions make sense.

Memory aid: 5P is one useful organizing mnemonic.

2. Describe the storming and norming stages of group development. (4mks)

- Storming: conflict, resistance, competition and testing of roles/leader.
- Norming: cohesion, shared norms, trust and cooperation become more stable.

Memory aid: *Storm -> Norm.*

3. Give four examples of counseling skills used in an intake session. **(2mks)**

- Examples: active listening, empathy, open-ended questions, reflection, paraphrasing, summarizing, clarification, therapeutic silence, appropriate confrontation.

Memory aid: *LEAP: Listen, Empathize, Ask, Paraphrase.*

SUPERVISED VOCATIONAL QUALIFICATION DIPLOMA - EXAMINATION

Session: Revision Mock Set 08	Duration: 1hr 30mins
Specialty: Psychotherapy	Coefficient: 04
Eliminatory mark: < 14/20	Training reference: VTI-AEDA examination style

SIMULATION OF REAL WORKING SITUATION PAPER - SOLUTION / MARKING GUIDE PRACTICE SET 08

Revision material based on the supplied VTI-AEDA 2025 examination format and the supplied psychotherapy course recordings. Not an official examination paper.

DETAILED MODEL ANSWERS - SIMULATION OF REAL WORKING SITUATIONS

SECTION I

1. Driving phobia - anchor: Behavior therapy - graded exposure/systematic desensitization, with CBT elements as needed

Yannick, 35, stopped driving after a serious road accident many years ago. As a passenger she develops sweating, shaking, light-headedness and an urge to escape, except when a trusted relative drives and talks calmly. A new job now requires independent travel and Yannick wants to regain driving confidence.

Model response / marking points (25 marks):

1. Assess the fear, accident memory, panic symptoms, avoidance, current driving competence and safety.
2. Explain the fear-avoidance cycle and rationale for exposure.
3. Teach a coping/relaxation skill without turning it into avoidance.
4. Build a hierarchy from low to high fear (e.g., sitting in parked car, engine on, empty road, short familiar route, busier road).
5. Use repeated in vivo graded exposure, remaining in each step long enough for learning.
6. Reduce safety behaviors gradually while keeping actual road safety.
7. Identify and test catastrophic thoughts such as "I will certainly crash" or "anxiety means danger."
8. Use homework and record anxiety/predictions/outcomes.
9. Reinforce progress and move up the hierarchy collaboratively.
10. Plan relapse prevention and future independent practice.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: RHE: Relax/coping, Hierarchy, Exposure - then Repeat and Review.

2. Depression after loss - anchor: Major depressive presentation - CBT with safety assessment and behavioral activation

For the past three weeks, Zita, 42, has felt sad nearly every day after losing a job. he reports loss of interest, fatigue, poor concentration, reduced appetite, insomnia and thoughts such as "I am useless and nothing will improve."

Model response / marking points (25 marks):

1. Recognize a depressive syndrome and assess duration, severity, impairment and differentials.
2. Ask directly about suicidal thoughts, plan, intent, means and protective factors; escalate/referral if risk is acute.
3. Collaboratively map situation-thought-emotion-behavior links.
4. Identify distortions such as overgeneralization, labeling and hopeless predictions.
5. Use evidence for/against and balanced alternative thoughts.
6. Introduce graded behavioral activation beginning with small achievable activities.

7. Address sleep/routine and social support.
8. Set SMART goals and a simple monitoring sheet.
9. Coordinate medical/psychiatric assessment when severity, suicidality or medication questions require it.
10. Review learning and homework.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: CBT TEA + SIGECAPS; safety first.

SECTION II

1. Bereavement and person-centered counseling - anchor: Person-centered therapy with grief assessment

Alice, 49, recently lost a spouse and arrives tearful, disheveled and unsure whether the intensity of grief is "normal." she says family members keep telling her to "move on," which has made her stop talking about the loss.

Model response / marking points (25 marks):

1. Create safety and let the client set the pace.
2. Use accurate empathy and reflective listening.
3. Offer unconditional positive regard/nonjudgment.
4. Be congruent/genuine without making the session about the therapist.
5. Invite the client to describe the meaning of the loss and present emotions.
6. Avoid premature advice, reassurance or forced positivity.
7. Assess functioning, supports and safety, including suicidal ideation where indicated.
8. Differentiate normal grief from a depressive disorder through duration, pervasive anhedonia/hopelessness and functional impairment without pathologizing grief automatically.
9. Help the client identify needs and supports.
10. Summarize and collaboratively decide next steps.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: EUC: Empathy, Unconditional positive regard, Congruence.

2. Generalized anxiety and worry - anchor: GAD - CBT with cognitive restructuring, worry management and relaxation

Bruno, 56, worries most days about finances, health, family and work. The worry has persisted for many months, is difficult to stop, and is accompanied by muscle tension, restlessness and poor sleep. he repeatedly seeks reassurance but remains anxious.

Model response / marking points (25 marks):

1. Assess breadth, duration and controllability of worry and rule out substance/medical contributors.
2. Formulate the cycle of threat prediction, reassurance/avoidance and short-lived relief.
3. Use psychoeducation about anxiety and uncertainty.
4. Identify probability overestimation and catastrophic beliefs.
5. Use cognitive restructuring and behavioral experiments.
6. Teach breathing or progressive muscle relaxation for arousal.
7. Consider scheduled worry/problem-solving time and reduction of reassurance seeking.
8. Set graded behavioral goals and homework.
9. Review sleep, caffeine/substance use and supports.
10. Monitor outcome and adjust plan.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: GAD = General Anxiety across Domains; CBT = Catch, Check, Change.